

HEAL-AI Website: Organizational Plan

SECTION 1: CURRENT WEBSITE STRUCTURE

The HEAL-AI website currently has six main pages. The following describes what each contains and its relevance to the video series.

1.1 Home

Overview of the HEAL-AI Lab's mission: identifying and addressing ethical issues from healthcare AI use before they become problems. Describes the team composition (bioethicists, physicians, lawyers, computer scientists) and the dual funding mandate (PCORI and Stanford Impact Labs) to refine the process for adoption by other health systems. Includes links to "what we do" and "how we can help."

Relevance to video series: Directly supports Topic 1 (Why This Matters). The urgency and mandate are clearly stated here, though the page currently reads as an institutional introduction rather than a call to action for external adopters.

1.2 Our Process

Describes Stanford's FURM (Fair, Useful, and Reliable Model) assessment framework, within which HEAL-AI's ethical assessments sit. Details the four-step process: (1) intake interview, (2) stakeholder interviewing (proposers, users, patients), (3) expert vetting, and (4) delivery of findings. Notes the process is designed to be completed within 30 days by a small team without deep technical ML expertise. Includes process diagrams.

Relevance to video series: The primary destination for Topic 2 (The Process Overview). Content is strong and well-structured. The coming-soon step-by-step summary will further reinforce this page.

1.3 Resources

The most content-rich page on the site. Currently organized into three areas:

- External video content: a four-part DiME (Digital Medicine Society) video series featuring Dr. Char on ethical and systemic challenges in deploying healthcare AI, plus two presentations from June 2025 (CHAI Leadership Summit; HAI Workshop).
- Ethics Assessment Reports: eight redacted sample reports from completed assessments, including HeartRead (hypertrophic cardiomyopathy screening), NoteBuddy (LLM nursing notes), AuthorizeMe (prior authorization AI), RadiRead (radiology report generation), Copilot (ambient scribe), Payment Probability & Denial Appeal Drafter, LabAlert (low-value lab reduction), and SendOff (readmission risk).
- Other Ethics Assessment Materials: a "What to Expect" document for proposer teams; a thematic content analysis template; a template for Expert Oversight Panel meetings; a stakeholder interview tracking sheet; and a metrics tool for assessing process usefulness. Several additional items are listed as coming soon: a detailed protocol for data collection,

a project manager's tracking sheet, a step-by-step process summary, a patient focus group moderator's guide, and interview guides for tool developers and prospective users.

Relevance to video series: Primary destination for Topics 4 and 6. Key materials for Topic 4 (interview guides) are listed as coming soon. The eight sample reports provide strong support for Topic 6.

1.4 Patient Partner Panel

Describes Stanford's own Patient Partner Panel: 10 volunteers recruited from the Patient and Family Partnership Program, representing diverse backgrounds. Details the training (in-person AI fundamentals), ongoing participation structure (4-person focus groups every couple of months), compensation, and the role of the moderator. Notes that after the ethics team drafts its report, all Panel members may review and suggest revisions.

Relevance to video series: Primary destination for Topic 5 (How to Build and Run a Patient Partner Group). The current page describes Stanford's specific implementation well, but does not yet serve as a practical how-to for institutions building their own equivalent. This is the most significant content gap for the video series.

1.5 People

Team member profiles. Not directly linked to any video topic, but establishes credibility and is likely to be visited by external institutions during due diligence.

1.6 Our Funders

Lists PCORI and Stanford Impact Labs as the primary funders. Provides institutional context. Not a destination for any video topic.

SECTION 2: VIDEO-TO-WEBSITE CONTENT MAPPING

Each video in the series is designed to introduce its topic and direct viewers to a specific section of the HEAL-AI website. The table below maps each topic to its website destination and the current readiness of that destination.

#	Video topic	Website destination	Status	Notes
1	Why This Matters	Home + Our Process (intro)	Content exists	Consider strengthening the Home page as a clearer entry point for external adopters

2	The Process Overview	Our Process (full page)	Content exists	Strong match. Step-by-step summary (coming soon) will reinforce
3	Adapting for Low-Resource Institutions	No corresponding page exists	Gap — must be built	New section needed under Resources or as a dedicated page: “Adapting the Process” or a Playbook page
4	How to Interview AI Developers and Clinicians	Resources → interview guides	Partial — coming soon	Thematic analysis template exists. Interview guide templates for developers and users are listed as coming soon and must be published before video launch
5	How to Build and Run a Patient Partner Group	Patient Partner Panel page	Partial — needs depth	Current page describes Stanford’s panel, not how another institution builds one. Moderator’s guide (coming soon) will help, but the page itself needs an external-facing how-to layer
6	From Focus Group to Final Report	Resources → reports and templates	Content exists	8 sample reports, analysis template, and EOP template are all live. Strong foundation. Walkthrough of a redacted report in the video should link to a specific report

Recommended action: Expand the Patient Partner Panel page to include a practical how-to guide for other institutions: recruitment approach, training structure, focus group facilitation, compensation considerations, IRB requirements, and how to scale down when full panel infrastructure is not feasible. The focus group moderator’s guide (listed as coming soon) should be published alongside the video.

SECTION 3: RECOMMENDED SITE STRUCTURE

The following reorganization is recommended to support the video series and the broader goal of making HEAL-AI's materials accessible to external adopters. Changes are incremental — no existing pages need to be removed, only expanded and re-organized.

3.1 Home page

The Home page should be sharpened as an entry point for external health systems, not just a description of the lab. Recommended additions:

- A clear statement of who the site is for: “If your health system is implementing AI tools and you want a structured way to assess them ethically, you’re in the right place.”
- A short “Start here” path that links to the video series, the Playbook (once built), and the Resources page.
- A sign-up prompt for updates (the Google Form linked from Resources should be more prominent).

3.2 Our Process

No structural changes needed. Publish the step-by-step process summary (currently coming soon) as a companion document, linked prominently from this page.

3.3 Resources

The Resources page currently mixes sample reports, external videos, templates, and publications without clear organization. As the content library grows alongside the video series, this will become harder to navigate. Recommended reorganization by process stage:

- Understanding the process — process overview, step-by-step summary, “What to Expect” doc, DiME video series
- Running stakeholder interviews — interview guides (developers, users), tracking sheet, thematic analysis template
- Running a patient partner group — moderator’s guide, focus group materials, IRB guidance
- Writing and delivering your report — EOP template, sample reports (all 8), metrics tool
- Adapting for your institution — Playbook, AI-assisted approaches, cross-institutional resources

Each section would include links to the corresponding video as its introduction, creating a natural loop between the video series and the resource library.

3.4 New page: Adapting the Process (or Playbook)

A new page to serve as the website destination for Topic 3. See Gap 1 above for content recommendations. This page is also the natural home for the Playbook mentioned in the Call to Action of the video brief. If the DiME Playbook is being referenced, it should be linked here with context. If HEAL-AI is developing its own Playbook, this page should host it.

3.5 Patient Partner Panel

Expand to serve both internal and external audiences. The current institutional description of Stanford's panel can remain as-is; add a second section titled "Building a panel at your institution" or "Adapting this model" with practical guidance for other health systems. This is the website destination for Topic 5 and should be ready to receive external visitors who want to replicate the model.

3.6 Training Videos (new section within Resources or standalone)

Once the video series launches, it needs a dedicated home on the site. Options:

- A "Training Videos" section within the Resources page, clearly separated from the external DiME series
- A standalone page in the navigation: Home / Our Process / Training Videos / Resources / Patient Partner Panel / People

The video section should list all six topics with brief descriptions, links to the videos, and links to the corresponding website resources — so that viewers can navigate the full series non-linearly and return to specific topics.

SECTION 4: NAVIGATION AND UX RECOMMENDATIONS

4.1 End-of-video calls to action should be specific

Each video should direct viewers to a named, stable URL — not just "visit our website." Coordinate exact URLs during pre-production so they can be included in on-screen graphics and show notes. For example:

- Topic 2 → heal-ai.stanford.edu/our-process
- Topic 4 → heal-ai.stanford.edu/resources (interview guides section)
- Topic 5 → heal-ai.stanford.edu/patient-partner-panel

Topics 1, 3, and 6 will need confirmed destinations based on new pages built per this plan.

4.2 Add a Get started flow for external organizations

External health systems arriving at the site for the first time whether from a video, a conference, or a referral currently face a navigation menu with no clear starting point. A simple entry path would reduce friction significantly. This could be a prominent section on the Home page or a lightweight “Get started” page:

1. Watch the introductory video series (link to video section)
2. Read about the process (link to Our Process)
3. Download the materials for your team (link to Resources, organized by stage)
4. Adapt the process for your institution (link to Playbook / Adapting the Process page)
5. Sign up for updates (link to Google Form)